

FLORA WELLNESS

Complete Guide Series | Volume 03

Your Hormone Type Blueprint

Discover Your Unique Hormone Profile &
Get a Personalized Protocol

Stop guessing — start targeting your exact imbalance

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SECTION 1: WHY HORMONE TYPING CHANGES EVERYTHING

The Problem with Generic Hormone Advice

'Take Vitex for hormones' is advice that helps one woman and can potentially worsen another. 'Avoid soy' is critical for some hormone types and irrelevant for others. 'Do more HIIT' is exactly right for an insulin-resistant woman and potentially harmful for an adrenal-burned-out woman. Generic hormone advice fails because hormone imbalance is not a single condition — it's a spectrum of distinct patterns, each with different root causes, different manifestations, and different solutions.

The Hormone Type Blueprint system identifies your primary hormonal pattern based on your specific symptom cluster, personal and family history, lifestyle factors, and (ideally) laboratory data. Once you know your type, every intervention becomes targeted and intentional rather than a scatter-gun approach. You stop wasting money on supplements that don't address your root cause and start getting results.

Most women have elements of multiple types — for example, cortisol dysregulation (Type 3) almost always creates some progesterone deficiency (Type 2) through the pregnenolone steal. The key is to identify your primary type (or the type that appeared first and is driving the others) and address that root imbalance, which often resolves the secondary imbalances downstream.

Hormone Type	Primary Driver	Most Common Symptoms	Most Affected Ages
Type 1: Estrogen Dominant	Excess estrogen relative to progesterone	Heavy periods, PMS, breast tenderness, bloating, weight gain	Late 20s through perimenopause
Type 2: Progesterone Deficient	Low progesterone from stress or anovulation	Anxiety, insomnia, luteal phase PMS, spotting, irregular cycles	Stress-prone women 25-45
Type 3: Cortisol Dysregulated	HPA axis overactivation or burnout	Fatigue, belly fat, anxiety, disrupted sleep, afternoon crash	High-achieving women, perimenopause

Hormone Type	Primary Driver	Most Common Symptoms	Most Affected Ages
Type 4: Thyroid Sluggish	Subclinical or clinical hypothyroidism, Hashimoto's	Fatigue, weight gain, hair loss, cold sensitivity, brain fog	30-55, more common after pregnancy
Type 5: Insulin Resistant	Impaired insulin signalling, blood sugar instability	Belly weight, sugar cravings, energy crashes, PCOS	20-50, increasing prevalence
Type 6: Androgen Excess	Elevated testosterone, DHEA, or DHT	Acne (jawline), excess hair, PCOS, oily skin, irregular cycles	Teens through 35, PCOS diagnosis

SECTION 2: THE HORMONE TYPE QUIZ

Instructions

Rate each symptom 0-3: 0 = Never/Not an issue | 1 = Occasionally | 2 = Often (most months) | 3 = Almost always or severely. Complete all sections before scoring. Be honest — rate how you feel NOW, not how you want to feel or used to feel.

PART A: MENSTRUAL CYCLE & REPRODUCTIVE SYMPTOMS

- Heavy or flooding periods (soaking pad/tampon in under 2 hours)
- Painful periods with cramping (rate 0-3 based on impact on function)
- Clots in menstrual blood
- Breast tenderness or lumpiness in the week before your period
- Bloating and water retention before period
- PMS mood swings, irritability, or rage in the week before period
- Spotting or brown discharge before period starts
- Periods shorter than 21 days or longer than 35 days
- Skipped periods (not pregnant) — score 3 if frequently
- Mid-cycle spotting or pain

Part A Score: ____/30

Score 15+: Strong Type 1 or Type 2 indicators. Score 8-14: Moderate hormonal disruption present. Continue to identify specific type.

PART B: ENERGY, FATIGUE & METABOLISM

- Fatigue that doesn't improve with sleep (rate severity 0-3)
- Energy crash at 2-4pm daily
- Wired but tired feeling — exhausted but can't wind down

- Needing caffeine to function (more than 1 cup/day)
- Unexplained weight gain or inability to lose weight despite effort
- Weight specifically accumulating around the belly and waist
- Feeling cold all the time, especially hands and feet
- Slow metabolism — feel like you gain weight just looking at food
- Sugar and carbohydrate cravings, especially in the afternoon
- Energy crash 1-2 hours after eating

Part B Score: ____/30

Score 15+: Likely Type 3 (cortisol), Type 4 (thyroid), or Type 5 (insulin). Refer to respective sections.

PART C: MOOD, BRAIN & NERVOUS SYSTEM

- Anxiety that worsens before your period or at specific cycle phases
- Depression or low mood that cycles with your period
- Brain fog or difficulty concentrating
- Forgetfulness or memory issues that feel significant
- Irritability, rage, or emotional dysregulation
- Panic attacks or feeling overwhelmed without clear cause
- Low motivation, drive, or sense of purpose
- Social withdrawal or reduced engagement
- Feeling 'not yourself' — disconnect from who you were
- Crying for no obvious reason

SECTION 3: TYPE 1 — ESTROGEN DOMINANT — FULL PROTOCOL

The Complete Estrogen Dominance Profile

Estrogen dominance doesn't necessarily mean you have too much estrogen — it means your estrogen-to-progesterone ratio is imbalanced in favor of estrogen. This can happen because estrogen is genuinely elevated (from xenoestrogens, impaired clearance, or fat cell production), or because progesterone is too low (from stress, anovulation, or perimenopause), or both simultaneously.

Estrogen dominance is arguably the most common hormonal pattern in women in industrialized countries, largely due to the ubiquitous presence of xenoestrogens in plastics, pesticides, personal care products, and synthetic hormones in the food supply. Add to this the widespread gut dysbiosis that impairs estrogen clearance, and liver stress from alcohol, medications, and processed foods, and you have a recipe for the estrogen dominance epidemic.

The Type 1 Supplement Protocol

Supplement	Mechanism	Dose	Timing	Notes
DIM	Shifts estrogen metabolism to 2-OH pathway (protective)	100-200mg	With largest meal	Take with fat; takes 4-6 weeks to see effect; stack with calcium D-glucarate
Calcium D-Glucarate	Inhibits beta-glucuronidase, prevents estrogen reabsorption	500mg	2-3x with meals	Critical if gut dysbiosis present; take consistently
Vitex	Stimulates LH, supports progesterone production to balance estrogen	400mg	Morning empty stomach	3-6 months to work; do not use with HBC

Supplement	Mechanism	Dose	Timing	Notes
Magnesium Glycinate	Supports Phase 2 liver detox; reduces PMS; progesterone support	300-400mg	Evening	Most women are deficient; very safe long-term
NAC (N-acetyl cysteine)	Liver glutathione precursor; supports Phase 2 detox	600mg	Away from meals	Morning or afternoon; most effective away from food
Milk Thistle (Silymarin)	Protects liver from damage; supports Phase 1 and 2 detox	150-300mg	With meals 2x	Use standardized extract 70-80% silymarin
Flaxseed (2 tbsp/day)	Lignans competitively bind estrogen receptors; modulates estrogen	2 tbsp ground	Daily with food	Always grind fresh; store in fridge
Sulforaphane	Activates NRF2 pathway; potent Phase 2 liver detox inducer	10-30mg extract or 50-100g sprouts	With food	Broccoli sprout powder or supplement

The Type 1 Nutrition Focus

- Daily cruciferous vegetables 1-2 cups (broccoli, kale, Brussels sprouts, cauliflower, arugula)
- 2 tablespoons freshly ground flaxseed daily — adds to smoothies, yogurt, oatmeal
- 1-2 cups pomegranate juice or seeds weekly (contains aromatase inhibitors)
- High fiber diet (35g+ daily) to ensure daily bowel movements for estrogen clearance
- Rosemary with meals (carnosol activates liver detox enzymes)

- Eliminate alcohol completely during 8-week reset (alcohol blocks estrogen clearance by up to 300%)
- Eliminate non-organic animal products (contain synthetic estrogen and growth hormones)
- Green tea 2-3 cups daily (EGCG inhibits aromatase and estrogen receptor activity)
- Avoid non-organic soy in large amounts (estrogenic phytoestrogens in excess can be problematic)
- Prioritize organic produce especially for the dirty dozen

SECTION 4: TYPE 2 — PROGESTERONE DEFICIENT — FULL PROTOCOL

Why Progesterone Deficiency Is an Epidemic

Progesterone deficiency has become one of the most common hormonal patterns in reproductive-age women, and chronic stress is the primary driver. Cortisol and progesterone share the same precursor (pregnenolone). When cortisol demand is chronically high — from work stress, relationship stress, financial stress, over-exercise, under-eating, or any other stressor — pregnenolone is diverted toward cortisol production, depleting the raw material available for progesterone.

A second major cause is anovulation — cycles where no ovulation occurs. Without ovulation, the corpus luteum (the temporary structure that produces progesterone in the second half of the cycle) never forms, meaning zero progesterone is produced in the luteal phase. Anovulatory cycles often look 'normal' from the outside — they still involve a period — but the hormonal profile is profoundly different and symptoms include anxiety, PMS, spotting, and luteal phase defect.

Confirming Low Progesterone

The key test is a mid-luteal progesterone blood test, taken on days 19-22 of your cycle (7 days after ovulation in a 28-day cycle, or approximately 1 week before your expected period). Optimal mid-luteal progesterone is 10-25 ng/mL. Values below 10 indicate low progesterone; below 5 suggests possible anovulation. Basal body temperature (BBT) tracking also confirms ovulation — you should see a clear 0.2-0.4°C temperature rise after ovulation that maintains through the luteal phase.

Supplement	Dose	Mechanism	Important Notes
Vitex / Chasteberry	400-600mg daily (morning, empty stomach)	Stimulates LH surge and pituitary dopamine receptors to increase progesterone	3-6 months for full effect; may initially alter cycle length; avoid in PCOS with high LH

Supplement	Dose	Mechanism	Important Notes
Magnesium Glycinate	400mg nightly	Reduces cortisol (preventing pregnenolone steal), supports progesterone pathway	Most critical deficiency for progesterone support; take consistently
Vitamin B6 P5P	50-100mg daily	Cofactor in progesterone synthesis; reduces estrogen dominance	Use P5P (active form); take with food to avoid nausea
Zinc	25-30mg with dinner	Required for LH surge triggering ovulation; without ovulation no progesterone	Take with food; pair with 1mg copper if supplementing long-term
Vitamin C	1000mg 2-3x daily	Corpus luteum is highest in vitamin C; supports progesterone production	Split doses throughout day; use food-based or buffered ascorbate
Bioidentical Progesterone Cream	20-40mg/day applied to skin	Direct progesterone supplementation during luteal phase	Only with practitioner guidance; use days 15-28 of cycle

SECTION 5: TYPE 3 — CORTISOL DYSREGULATED — FULL PROTOCOL

The Three Stages of HPA Dysregulation

HPA axis dysregulation progresses through identifiable stages. Understanding which stage you're in determines which interventions will help — and which might worsen your situation.

Stage	Description	Key Signs	Testing	Protocol Priority
Stage 1: Hyperactivation	Chronically elevated cortisol, high output	Anxiety, insomnia, wired feeling, fast heartbeat, high blood pressure, cravings, belly fat	High morning cortisol (>20 mcg/dL), elevated evening cortisol, flat curve	Cortisol-reducing adaptogens, parasympathetic practices, no HIIT
Stage 2: Compensated	Cortisol output becomes erratic, rhythm disturbed	Wired-and-tired (tired but can't sleep), afternoon crash, disrupted sleep, brain fog	Low morning/high evening cortisol or blunted peaks, DHEA starting to drop	Blood sugar stabilization, sleep protocol, adaptogens, rhythm restoration
Stage 3: Burnout	Cortisol production depleted, low output	Complete exhaustion, salt cravings, dizziness on standing (POTS-like), zero motivation, crashes from any exercise	Flat diurnal curve, low morning cortisol (<5 mcg/dL), very low DHEA-S	No adaptogens that are stimulating, rest, nutrition, slow re-building, NOT aggressive detox

Breathwork Protocols for Immediate Cortisol Reduction

Physiological Sigh (fastest cortisol drop)

Two inhales through the nose (first a full inhale, then a second 'top-up' sniff to fully inflate the lungs), followed by a long, slow exhale through the mouth. Repeat 1-5 times. Research shows this single

technique produces the fastest reduction in physiological stress markers of any breathing technique.

Box Breathing (nervous system regulation)

Inhale through nose for 4 counts, hold for 4 counts, exhale through mouth for 4 counts, hold empty for 4 counts. Complete 4-8 rounds. Used by Navy SEALs to down-regulate under extreme stress. Practice 2x daily for cumulative cortisol-lowering effects.

4-7-8 Breathing (sleep and anxiety)

Inhale through nose for 4 counts, hold for 7 counts, exhale through mouth for 8 counts. Complete 4 cycles. The extended exhale activates the parasympathetic nervous system strongly. Best used before sleep or during acute anxiety.

Coherent Breathing (HRV optimization)

5 breaths per minute: inhale for 6 counts, exhale for 6 counts. Continue 10-20 minutes. This rate maximizes heart rate variability (HRV), a direct measure of vagal tone and stress resilience. The single most evidence-based breathing practice for long-term HPA normalization.

SECTION 6: TYPE 4 — THYROID SLUGGISH — FULL PROTOCOL

Understanding Thyroid Function Beyond TSH

The standard TSH (thyroid stimulating hormone) test alone catches only the most severe cases of thyroid dysfunction. TSH can be 'normal' (2.5-3.0) while a woman has debilitating hypothyroid symptoms, because standard lab ranges include the outer 2.5% of a population that includes many sick people. Functional medicine optimal TSH is 0.5-2.0, and most women feel best under 1.5.

The full thyroid picture requires: TSH (the pituitary signal), Free T4 (total thyroid output), Free T3 (the active, metabolically usable form), Reverse T3 (inactive T3 that blocks T3 receptors), TPO antibodies (Hashimoto's marker), and Thyroglobulin antibodies (second Hashimoto's marker). This complete panel often reveals patterns that TSH alone misses — particularly elevated reverse T3 from chronic stress, and positive antibodies indicating autoimmune thyroiditis years before TSH becomes abnormal.

Test	Optimal Range	What It Reveals	What Disrupts It
TSH	0.5-2.0 mIU/L	Overall pituitary-thyroid communication	Stress, inflammation, fasting, selenium deficiency
Free T4	1.1-1.5 ng/dL	Total thyroid hormone production	Iodine deficiency, selenium deficiency, autoimmune attack
Free T3	3.2-4.2 pg/mL	Active thyroid hormone available to cells	Cortisol blocking conversion, selenium/zinc deficiency, inflammation
Reverse T3	<15 ng/dL	Inactive T3 blocking T3 receptors — a parking problem	Chronic stress is #1 cause; crash dieting, trauma, illness

Test	Optimal Range	What It Reveals	What Disrupts It
TPO Antibodies	<35 IU/mL	Autoimmune attack on thyroid (Hashimoto's)	Gluten, leaky gut, infections, iodine excess, environmental toxins
Thyroglobulin Ab	<40 IU/mL	Second Hashimoto's marker (catches more cases than TPO alone)	Same triggers as TPO antibodies

SECTION 7: TYPE 5 — INSULIN RESISTANT — FULL PROTOCOL

The Insulin-Hormone Connection

Insulin resistance doesn't just cause weight gain and blood sugar problems — it is a profound hormonal disruptor that can trigger a cascade affecting every other hormone system. In the ovaries, excess insulin directly stimulates androgen (testosterone) production, causing PCOS in genetically susceptible women. Elevated insulin also drives inflammation, suppresses SHBG (sex hormone binding globulin, which controls free hormone levels), and impairs progesterone production.

Insulin resistance exists on a spectrum: you can have significantly impaired insulin signalling for years or decades before blood sugar rises to diabetic levels. The earliest markers appear in fasting insulin (optimal: 2-8 mIU/L; most labs consider up to 20 'normal'), followed by elevated triglycerides, low HDL, increased waist circumference, and elevated fasting glucose. The HOMA-IR calculation ($\text{fasting glucose} \times \text{fasting insulin} \div 405$) is the best single metric: optimal is under 1.5.

Intervention	Mechanism	Implementation	Timeline
Post-meal walking	Muscle glucose uptake independent of insulin signalling	10 min walk within 30 min of main meals	Effect seen after first meal
Berberine 500mg 3x with meals	Activates AMPK like metformin; improves insulin receptor sensitivity	500mg with each meal, take for 3-6 months	4-8 weeks for significant effect
Myo-inositol 4g + D-chiro-inositol 100mg	Second messenger in insulin signalling; especially critical for PCOS	Split dose morning and evening; 40:1 myo:D-chiro ratio	3-6 months for PCOS benefit
Chromium 400-600mcg	Potentiates insulin receptor activity; reduces carb cravings	With main meals	2-4 weeks for craving reduction

Intervention	Mechanism	Implementation	Timeline
Magnesium 400mg	Cofactor in insulin receptor kinase; required for glucose transport	Evening	Immediate improvement in insulin signalling
Time-restricted eating 12-14 hours	Reduces insulin exposure, allows metabolic flexibility	Stop eating by 7-8pm, don't eat until 8-9am; no longer window needed	2-4 weeks

SECTION 8: TYPE 6 — ANDROGEN EXCESS — FULL PROTOCOL

Androgen Excess in Women: Beyond PCOS

Androgen excess presents differently than in men — instead of very high testosterone, women typically see relatively modest elevations in free testosterone, DHEA-S, or androstenedione that are enough to cause significant symptoms due to our greater estrogen/androgen sensitivity. The most common cause is PCOS (polycystic ovary syndrome), which affects 8-13% of reproductive-age women and is driven by insulin resistance, inflammation, and HPA dysregulation — not primarily an ovarian problem.

Symptom	Hormonal Driver	Most Effective Natural Intervention
Jawline/chin acne	Elevated testosterone, DHT	Spearmint tea 2x daily (reduces free testosterone); zinc 30mg; inositol for PCOS
Excess facial/body hair (hirsutism)	Elevated DHT from testosterone	Saw palmetto 320mg (5-alpha reductase inhibitor); spearmint tea; address insulin resistance
Oily skin	DHT stimulating sebaceous glands	Zinc, spearmint tea, inositol; address root insulin/PCOS driver
Irregular or absent periods	LH/FSH imbalance, chronic anovulation	Inositol 4g daily; address insulin resistance; ashwagandha for LH normalization
Scalp hair thinning (female pattern)	DHT sensitivity at hair follicle	Saw palmetto, zinc, address thyroid and iron simultaneously
Mood changes, aggression	Testosterone fluctuations	Magnesium, B6, regular exercise, stress reduction

SECTION 9: MIXED TYPES & COMPLEX PATTERNS

The Most Common Mixed Type Patterns

Type 1 + Type 2 (Estrogen Dominant + Low Progesterone)

The most common combination. Estrogen is elevated OR excess relative to low progesterone. Both hormones are imbalanced in the same direction (estrogen dominance) but from different ends. Often caused by chronic stress simultaneously causing progesterone deficiency while impaired liver detox allows estrogen accumulation. **Priority:** Address both simultaneously: liver detox + Vitex + stress reduction. DIM helps both by improving estrogen clearance and slightly increasing progesterone through reduced estrogenic competition.

Type 2 + Type 3 (Low Progesterone + High Cortisol)

Extremely common — cortisol directly causes progesterone deficiency through the pregnenolone steal. Addressing cortisol always comes first here: if you only add progesterone support (Vitex) without fixing the cortisol, you're adding water to a leaking bucket. **Priority:** Address cortisol first (6-8 weeks of adrenal support), then add progesterone-specific support. Magnesium glycinate addresses both simultaneously.

Type 3 + Type 4 (High Cortisol + Sluggish Thyroid)

Cortisol directly blocks T4 to T3 conversion, creating functional hypothyroidism even with a structurally normal thyroid. Women in this pattern often don't respond well to thyroid medication alone because the cortisol is continuously blocking conversion. **Priority:** Must reduce cortisol to restore thyroid function. Standard thyroid medications may help but won't be optimal until cortisol is managed. This pattern often normalizes completely once cortisol is addressed.

Type 3 + Type 5 (Cortisol + Insulin Resistance)

Cortisol raises blood sugar (glucose production) while also causing insulin resistance in peripheral tissues. This creates a double hit: more glucose AND less efficient glucose clearance. Weight loss is nearly impossible in this pattern until both are addressed. **Priority:** Start with blood sugar stabilization (immediate) and cortisol reduction (medium-term). Post-meal walks address both simultaneously. Both berberine and ashwagandha help both conditions.

SECTION 10: BUILDING YOUR PERSONALIZED PROTOCOL

The 5-Step Protocol Building Process

Step 1: Confirm Your Type with Lab Testing

Identify your primary type from the quiz, then confirm with targeted lab work. At minimum: mid-luteal E2, progesterone, testosterone (free and total), DHEA-S, morning cortisol, fasting insulin, and full thyroid panel including antibodies. This takes the guesswork out entirely.

Step 2: Remove Your Specific Triggers

Different types have different primary triggers to eliminate first. Type 1: xenoestrogens and alcohol. Type 2: stressors and under-eating. Type 3: cortisol drivers (work, training load, blood sugar swings). Type 4: gluten (if Hashimoto's), fluoride, goitrogens. Type 5: refined carbs, sedentary habits.

Step 3: Add Targeted Nutritional Support

Type-specific foods and macronutrient ratios. Type 1 and 2 need high fiber and cruciferous. Type 3 needs blood sugar stability and adrenal nutrients. Type 4 needs selenium, iodine, and anti-inflammatory foods. Type 5 needs low glycemic, high protein, strategic carb timing.

Step 4: Implement Your Supplement Protocol

Start with universal foundation (magnesium, omega-3, vitamin D), then add type-specific supplements in the order and doses specified in your type's section. Don't add everything at once — start one new supplement per week to identify what's helping.

Step 5: Retest and Adjust at 3 Months

This is where most women skip a critical step. Retesting hormones at 3 months shows objective improvement (or lack thereof), guides supplement adjustments, and identifies any secondary imbalances that have emerged. Mid-luteal blood test or DUTCH test is ideal.

SECTION 11: HORMONES THROUGH LIFE STAGES

Life Stage	Dominant Pattern	Common Symptoms	Key Interventions
Teens (13-19)	Establishing hormonal rhythm; androgens higher; estrogen building	Irregular cycles, acne, PMS, mood instability	Seed cycling, zinc, magnesium, inositol (if PCOS), no restrictive dieting
20s	Cortisol and estrogen often high from lifestyle stress; insulin resistance building	PMS, fatigue, irregular cycles, PCOS onset	Stress management is primary; address gut, liver, and insulin
30s	Progesterone begins declining; cortisol often elevated; fertility concerns	PMS worsening, anxiety, weight creep, perimenopause early signs in some	Prioritize progesterone support, stress reduction, thyroid testing
Early 40s	Perimenopause begins: erratic estrogen, falling progesterone, testosterone declining	Hot flashes, sleep disruption, mood changes, irregular cycles, weight gain	Comprehensive hormone support; adrenal health critical; consider BHRT with practitioner
Late 40s-50s	Menopause transition: estrogen declining permanently, all sex hormones lower	Hot flashes, vaginal changes, bone density concerns, cognitive changes	BHRT consideration, phytoestrogens, bone health protocol, cardiovascular support
50s+	Post-menopause: new hormonal steady state	Bone density, cardiovascular, cognitive, vaginal health	Long-term maintenance; BHRT if appropriate; continue hormone-supportive lifestyle

ADVANCED TESTING & RESOURCES

The DUTCH Test Explained

The DUTCH (Dried Urine Test for Comprehensive Hormones) test is the most comprehensive hormone test currently available for women. Unlike blood tests that capture a single time-point snapshot, DUTCH captures hormone metabolites across 24 hours using dried urine samples collected at specific times. This provides not only hormone levels but also metabolite patterns showing HOW your body is processing hormones.

DUTCH reveals: total estrogen production AND metabolite pathway (2-OH vs 4-OH vs 16-OH estrogen), progesterone and its metabolites, testosterone and DHT, DHEA and its metabolites, cortisol throughout the day and melatonin. For estrogen specifically, DUTCH shows whether you're making too much, not clearing it well, or sending it down harmful metabolic pathways — information that blood tests simply cannot provide.

DUTCH Complete

When: Best 2-3 years into hormone concerns; before prescribing BHRT **Reveals:** Full picture of hormone production, metabolism, cortisol rhythm, and melatonin

GI-MAP (comprehensive stool test)

When: When gut symptoms present alongside hormone issues **Reveals:** Pathogens, beta-glucuronidase levels, microbiome diversity, leaky gut markers

Genetic testing (23andMe + Rhonda Patrick analysis)

When: Anytime; permanent data **Reveals:** COMT gene (estrogen metabolism), MTHFR (methylation), BRCA (risk), CYP1B1 (estrogen pathway)

Organic Acids Test (OAT)

When: When energy, mood, and brain fog are primary **Reveals:** Mitochondrial function, B vitamin status, yeast/bacterial overgrowth, neurotransmitter metabolism